

ST. JUDE METROPOLITAN HOSPITAL - CLINICAL RECORD

CONFIDENTIAL MEDICAL INTELLIGENCE REPORT

POST-OPERATIVE SURGICAL CONSULTATION

Patient Name: Samuel Sterling

Age: 74 | Gender: Male | MRN: #SS-551902

Date: 2026-08-25 | Location: Surgical ICU Bed 04

Surgeon: Dr. Kenneth Holloway, MD (Orthopedic Surgery)

PROCEDURE:

Day 2 Post-Op Right Total Hip Arthroplasty (THA).

CLINICAL EVALUATION:

Patient reports worsening right calf tenderness, swelling, and mild pleuritic chest discomfort. Surgical incision is clean, intact with minimal serosanguinous drainage.

VITALS:

BP: 148/88 mmHg | HR: 104 bpm (Tachycardic) | RR: 20 bpm | Temp: 38.0 C | SpO2: 93% on room air (Down from 98%)

LABS & DIAGNOSTICS:

- D-Dimer: 3.4 mcg/mL FEU [HIGH] (Reference: < 0.5 mcg/mL)
- Hemoglobin: 10.2 g/dL [LOW] (Stable from post-op baseline)
- Platelet Count: 240 x10³/uL [NORMAL]

CURRENT MEDICATIONS:

- Enoxaparin 40 mg SC Daily (Prophylaxis)
- Hydromorphone 2 mg IV PRN severe pain
- Cefazolin 1g IV q8h (Completed 24h post-op course)

RISK ASSESSMENT:

High suspicion for Deep Vein Thrombosis (DVT) and potential Pulmonary Embolism (PE).

ACTION PLAN:

1. Priority 1 (Radiology / Physician): Stat CT Pulmonary Angiography (CTPA) and bilateral lower extremity venous duplex ultrasound.
2. Priority 2 (ICU Staff): Place patient on 2L supplemental oxygen via nasal cannula to maintain SpO2 > 95%.
3. Priority 3 (Pharmacy): Prepare therapeutic anticoagulation pending imaging confirmation.